Sutter Health

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June 16, 2025

Dr. Mehmet Oz
Administrator
Centers for Medicare & Medicaid Services

Dr. Thomas Keane, M.D., M.B.A.
Assistant Secretary for Technology Policy & National Coordinator
for Health IT
U.S. Department of Health and Human Services

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services

RE: Request for Information: Health Technology Ecosystem (CMS-0042-NC) [RIN 0938-AV68]

Dear Secretary Kennedy, Administrator Oz, and Dr. Keane:

On behalf of Sutter Health, we appreciate the opportunity to respond to the Request for Information issued by the Department of Health and Human Services (HHS), the Centers for Medicare & Medicaid Services (CMS), and the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC).

Sutter Health is a not-for-profit health care system dedicated to providing comprehensive care throughout California. Committed to community partnerships and innovative, high-quality patient care, Sutter Health is pursuing a bold new plan to reach more people and make excellent health care more connected and accessible. Sutter Health currently serves nearly 3.5 million patients, thanks to a dedicated team of more than 57,000 employees and clinicians and 12,000+ affiliated physicians, with a unified focus on expanding care to serve more patients. Sutter delivers exceptional and affordable care through its hospitals, medical groups, ambulatory surgery centers, urgent care clinics, telehealth, home health, and hospice services.

Sutter is deeply invested in using technology to improve health outcomes. As a leader in value-based care, Sutter supports CMS, HHS, and ONC's efforts to align regulatory frameworks and payment models with technologies that enhance clinical outcomes, expand equitable access, reduce burden, expand interoperability, empower patients and providers while protecting sensitive data.

We are pleased to offer our program level insights and broader structural recommendations on topics including digital identity, interoperability, and provider burden reduction.

1. Digital Health Applications and Provider Adoption

PR-2. Obstacles in Physician Workflows

A persistent barrier to the development and effective utilization of innovative applications for physician workflows (such as quality reporting, clinical documentation, and billing) is the complexity and fragmentation of existing health IT systems. Many healthcare organizations must maintain large internal IT teams to manage disparate platforms, custom interfaces, and siloed data repositories. As a result, even high-value digital tools often face significant adoption hurdles, particularly in resource-constrained settings.

It is significantly easier and more efficient to integrate new functionalities into a single, unified EHR platform than to manage a multitude of separate applications. However, the current lack of standardized integration frameworks, inconsistent data definitions, and variable clinical workflows across vendors severely limits scalability.

CMS can mitigate these challenges by promoting standards-based integration directly within EHR platforms. Encouraging the adoption of standardized data formats, data quality protocols, and clearly defined use cases would allow for faster and more consistent implementation of digital tools. Reducing variability and ambiguity in data expectations would enable developers and providers alike to focus resources on innovation, rather than custom integration.

PR-3. Importance of EHR Data Accessibility

Interoperable data is foundational to supporting workflows and reducing provider burden. This requires targeted support for data quality improvement and standardization, which directly reduces the need for costly and time-consuming data mapping across disparate systems. One key opportunity is to invest in initiatives that promote consistent coding (e.g., LOINC, SNOMED), structured clinical documentation, and adherence to national interoperability standards. Machine learning may also help reduce the mapping and transformation workload, but its effectiveness depends on a strong foundation of high-quality source data. Reducing the burden of integration will allow programs to focus more resources on patient engagement and proactive clinical intervention, rather than on technical infrastructure maintenance.

PR-9. Encouraging Adoption of Digital Identity Credentials

Digital identity is a critical foundation for secure and streamlined health data exchange, particularly as more patients engage with digital health tools across multiple providers and platforms. CMS can play a pivotal role in accelerating provider adoption of trusted digital identity credentials, such as CLEAR, Login.gov, or ID.me, by addressing key operational and regulatory barriers.

a. What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?

Digital identity should be understood as three distinct components: identity verification, patient matching, and authorization. While Credential Service Providers (CSPs) can effectively verify a patient's identity, they cannot—and should not—control patient matching or access

permissions. These steps must remain under the control of covered entities to ensure compliance with HIPAA and ethical stewardship of protected health information.

For identity verification, Sutter supports an open, innovation-driven ecosystem that allows providers flexibility in how they implement secure identity verification solutions. Rather than mandating a single identity provider, CMS should promote standards-based credentialing practices that integrate with existing provider workflows and health IT systems.

b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?
A trusted digital identity framework must be voluntary, secure, and interoperable. Patients should be clearly informed about how their credentials will be used and how data access is managed across systems. CMS can enhance adoption by supporting "single sign-on" capabilities that allow patients to authenticate once and use their credentials across health plans, provider portals, and approved third-party applications.

PR-10. Implementation and Implications of Digital Identity Credentials
a. What are the challenges and benefits for providers?
Challenges include lack of standardization among credentialing services, security concerns, potential liability issues, and variability in patients' ability to access necessary technology or required documents. However, benefits include enhancing clinical and administrative efficiency by reducing risk of identity fraud, improving patient matching, and streamlining onboarding processes.
b. How would requiring their use improve access to health information?
Verified digital credentials could reduce the need for multiple logins and help ensure continuity of access across provider systems and applications. However, CMS must recognize that identity verification alone does not guarantee appropriate data access. Authorization workflows must remain locally governed to protect patient rights and maintain accountability.

c. What are the potential downsides?
Mandatory credentialing could unintentionally exclude vulnerable populations, such as patients without access to mobile devices or necessary documentation. Additionally, patients may not fully understand the implications of credential sharing, which could increase privacy risks. Authorization must remain the responsibility of the covered entity to ensure appropriate oversight and avoid unintended disclosures or misuse of sensitive health information.

d. What impact would mandatory credentials have on a nationwide provider directory?
Mandatory digital credentials could improve the security and accuracy of interactions with a national provider directory by enabling authenticated access and reducing the likelihood of fraudulent submissions. However, this should not come at the cost of increased burden for smaller providers or under-resourced organizations.

e. How could digital identity implementation improve provider data flow?
Standardized digital credentials can enhance identity matching and consent tracking—both of which are critical to seamless, secure exchange of patient information. This supports more efficient coordination of care across settings and systems.

f. Would combining FHIR addresses and identity improve data flow?
Yes. Linking validated digital credentials with FHIR endpoint directories could streamline routing, improve traceability, and facilitate interoperability by connecting patients to the correct provider access points.

PR-14. Encouraging Use of the Information Blocking Portal
Sutter Health believes CMS and ONC should prioritize collaboration and clarity over punitive enforcement when addressing information blocking. Today, providers often face a regulatory paradox: disclosing information too quickly may violate HIPAA, while holding it for contextual review risks being labeled as information blocking. These unclear and conflicting expectations generate significant compliance anxiety.
Sutter Health, which exchanges millions of medical records annually, is committed to responsible health data sharing and supports the goals of increased transparency and interoperability. However, the current enforcement approach—focused heavily on punitive action—risks discouraging good-faith participation and slowing progress.

CMS should consider allowing a remediation window for good-faith efforts before enforcement actions are taken. Such an approach would allow provider organizations to course-correct without fear of immediate penalties. A more collaborative regulatory environment would better support the goals of expanded interoperability.

Additionally, CMS and ONC should explicitly delineate the responsibilities of different actors—such as providers, EHR vendors, and HIEs—regarding compliance. Clear role-based expectations will help avoid confusion, improve accountability, and encourage more consistent adherence to information sharing requirements. In this way, reporting can advance data exchange by clarifying barriers and surfacing real-world issues rather than deepening mistrust in the regulatory process.

2. Digital Health in Value-Based Care Models

VB-1. Incentives to Drive ACO/MSSP Adoption
Targeted financial and/or quality-based incentives could be designed to directly support investment in and the use of digital health technologies that demonstrably advance value-based care goals—such as remote patient monitoring, telehealth for specialty access, secure messaging, and patient portals with advanced functionality.

In parallel, CMS could reduce administrative burden by allowing digital tools to facilitate more efficient, consolidated, and potentially automated data collection and reporting for state and federal programs. This would help align digital innovation with regulatory compliance and free up resources for direct patient care.

VB-3. Essential Health IT Capabilities
Standardized data definitions and machine-readable quality metrics are essential to ensure consistent performance measurement and reduce reporting friction across these tools. Ubiquitous and reliable patient event notifications are also critical, enabling timely care coordination and sustained patient engagement.

3. Technology Vendors, Data Providers, and Networks

TD-5. Nationwide Provider Directory of FHIR Endpoints

Sutter Health strongly supports the development of a nationwide provider directory of FHIR endpoints as a foundational infrastructure investment to improve access to health information for patients, providers, and payers. To be effective, this directory must serve as a single source of truth—consolidating, not duplicating, existing efforts across federal, commercial, and regional systems. It must also include FHIR endpoints at both the organization and individual provider levels, rather than limiting functionality to high-level or institutional endpoints.
Current tools such as the National Plan and Provider Enumeration System (NPPES) are insufficient for this purpose and must be significantly enhanced or replaced. A modernized directory should reflect the operational reality of interoperable care delivery and support clinical data exchange with specificity at the point of care.

A centralized, accurate directory would enhance care coordination by streamlining referrals, accelerating provider-to-provider communication, and supporting more efficient data exchange workflows. It would also reduce administrative burden by simplifying credentialing, contracting, and payment processing.

Responsibility for maintaining the directory should be shared across stakeholders. Payers should be accountable for updating panel and coverage information, while providers should maintain details related to practice locations and FHIR endpoint accessibility. Importantly, this burden must not fall to individual physicians or frontline staff but rather be integrated into existing organizational infrastructure.

Ultimately, well-designed and governed national provider directory can serve as a critical enabler of interoperability, quality reporting, and patient-centered care delivery at scale.

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Sutter Health appreciates the opportunity to provide input into the future of the digital health ecosystem. We are committed to collaboration with CMS, ASTP, and other stakeholders to advance interoperability, reduce administrative burden, and improve patient care through better use of health IT. We urge CMS to pursue policies that reduce fragmentation, support standardization, and allow health systems to innovate on a strong foundation of trusted, high-quality data exchange.

Should you have questions, please do not hesitate to contact Sutter Health's Vice President of Federal Policy & Regulatory Affairs, Jonathan Williams, at 916-718-2085 or email at Jonathan.Williams@sutterhealth.org.

Sincerely,

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Sutter Health